
Project Charter

Network Infrastructure Rollout – New Jane Furse Memorial Hospital Sekhukhune District Municipality, Limpopo

1. Project Purpose and Justification

This charter authorizes the design and implementation of a dedicated network infrastructure link between the Sekhukhune District Municipality and New Jane Furse Memorial Hospital. The project exists to close the digital divide affecting the district's largest public hospital, replacing unreliable, informal connectivity with a secure, sustainable network that can support clinical, administrative and supply-chain operations. It gives the project team a mandate to proceed, defines the boundaries of the work, and sets out how success will be measured over the twelve-month delivery window.

2. Project Objectives

Deliver a dedicated fibre connection from the municipal network to the hospital via Openserve within 12 months.

Provide secure, reliable network coverage to all 20 wards and 5 administrative offices.

Resolve the hospital's data storage and missing patient file concerns through a proper server and backup solution.

Equip the 3-person ICT team with the skills needed to operate and maintain the network independently.

Deliver the project within an approved budget of approximately R 1,342,000, plus contingency.

3. Scope

In Scope

Fibre splice and last-mile connection from the municipality to the hospital.

Internal LAN/Wi-Fi cabling, core network hardware, and VLAN segmentation.

Security infrastructure: firewall, VPN, endpoint protection, and physical security for the server room.

Local server and backup solution for patient records and other critical data.

Replacement of unusable legacy computers and two additional printers.

Training of the existing 3 ICT support staff.

Out of Scope

Replacement of the hospital's clinical, finance or supply-chain software applications.

Connectivity to satellite clinics or facilities outside the main hospital site.

Long-term managed services beyond the initial warranty/support period, unless separately contracted.

4. Key Stakeholders

Sekhukhune District Municipality – project sponsor, owner of the source network.

New Jane Furse Memorial Hospital management – client site, operational approval.

Hospital ICT support staff (3) – co-implementers and long-term operators.

Openserve – last-mile carrier and fibre infrastructure partner.

Hospital clinical and administrative staff – end users.

Suppliers using Intralink services – external users of hospital network services.

5. Key Milestones and Implementation Plan

The project is planned over a twelve-month period, structured into eight sequential phases. Each phase closes with a milestone sign-off before the next phase begins, giving the sponsor clear checkpoints to confirm progress and budget alignment.

Phase	Key Activities	Timeframe	Milestone
Phase 1	Project initiation, stakeholder sign-off, site survey, Openserve route survey	Month 1	M1
Phase 2	Detailed network design, procurement of fibre, hardware and security equipment	Months 2–3	M2
Phase 3	Trenching and fibre splice from municipality to hospital; carrier commissioning	Months 4–6	M3
Phase 4	Internal LAN/Wi-Fi cabling, core switch, firewall and server installation	Months 6–8	M4
Phase 5	Security hardening, backup configuration, hardware replacement for legacy PCs	Months 8–9	M5
Phase 6	System integration testing, load testing, penetration testing	Month 10	M6

Phase 7	ICT staff training (installation, troubleshooting, unit replacement)	Month 11	M7
Phase 8	Go-live, handover, warranty period begins, project closure	Month 12	M8

6. Key Deliverables

Deliverable	Description
Network design document	As-built and as-designed diagrams covering the fibre link, LAN, VLANs and security architecture
Dedicated fibre link	Splice from the municipal network to the hospital, commissioned and tested by Openserve
Hospital LAN & Wi-Fi	Structured cabling and wireless coverage across all 20 wards and 5 offices
Security infrastructure	Firewall, VPN, endpoint protection, access control and CCTV for the server room
Server & backup solution	Local file server with scheduled backups to resolve missing patient file records
Refreshed end-user devices	Replacement of unusable legacy PCs and two additional printers
Trained ICT team	3 ICT support staff certified on installation, troubleshooting and unit replacement
Operations & maintenance manual	Documented procedures for day-to-day support and escalation
Project closure report	Sign-off document confirming scope delivered, budget spent and lessons learned

7. Roles and Responsibilities

Project Sponsor (Municipality): approves budget, provides network access rights, resolves escalations.

Project Manager: owns the schedule, budget, and risk register; reports monthly to the sponsor.

Openserve: designs and installs the last-mile fibre link; provides ongoing carrier support.

Implementation Team/Contractor: installs internal cabling, hardware, security systems and servers.

Hospital ICT Support Staff: co-participate in installation, receive training, and take over day-to-day operations.

Hospital Management: coordinates site access, staff communication, and operational readiness.

8. Recommendations

Prioritize security hardening (firewall, access control) early, before broad network access is enabled, given the current absence of controls.

Negotiate a formal service-level agreement with Openserve covering uptime, response time and escalation, since it is the sole carrier.

Run the hardware audit in Phase 1 so procurement of replacement PCs happens in parallel with cabling, not after it.

Build in at least one refresher training session 2–3 months after go-live, once ICT staff have hands-on experience with real faults.

Establish a simple maintenance contract or retainer with Openserve or an implementation partner for the first 12–24 months post-handover, given the small internal ICT team.

Involve hospital staff and municipal representatives early and regularly, recognizing that the network's long-term viability depends on social as well as technical buy-in.

9. Risk Register

Risk	Likelihood	Impact	Mitigation
Small ICT team cannot sustain network after handover	High	High	Structured training plan, vendor support-desk contract, clear runbooks
Single-carrier (Openserve) dependency causes outage	Medium	High	SLA with defined uptime/response targets; secondary failover link if budget allows
Missing security controls exploited before hardening complete	Medium	High	Deploy firewall and access control early in Phase 3, before full rollout
Trenching delays due to permits/road crossings	Medium	Medium	Early engagement with municipality and road authority; buffer in schedule
Legacy hardware cannot run required software	Medium	Medium	Early hardware audit in Phase 1; phased replacement budget

Low staff buy-in / resistance to new systems	Medium	Medium	Stakeholder workshops, change-management communication, early staff involvement
Budget overrun	Medium	Medium	10% contingency, phased procurement, monthly budget tracking
One-year timeline slippage	Medium	High	Monthly milestone reviews, escalation path, realistic phase buffers

10. Assumptions and Constraints

Openserve remains available and willing to act as the sole last-mile carrier for the full project duration.

The municipality's existing network has sufficient capacity to be spliced without requiring its own upgrade.

Budget of approximately R 1,342,000 is approved and released in phases aligned to the implementation plan.

Hospital operations continue throughout installation, requiring work to be scheduled around clinical activity.

The 3 ICT support staff are available for training and are not reassigned during the project period.

11. Success Criteria

Dedicated fibre link operational and tested between the municipality and hospital within 12 months.

All 20 wards and 5 offices have verified network and Wi-Fi coverage.

Security controls (firewall, access control, backup) are in place and tested before go-live.

All 3 ICT staff successfully complete training and can independently perform basic troubleshooting and unit replacement.

Project delivered within the approved budget and the twelve-month timeframe, with formal sign-off from the district municipality and hospital management.

12. Approval

This charter authorises the project team to proceed with planning and implementation as described above. Formal approval by the Sekhukhune District Municipality and New Jane Furse Memorial Hospital management is required before Phase 1 activities commence.